



Intake Note

Counseling World

Clinician: Martha L Sanchez, LMSW

Supervisor: Kirsten Hockemeier

Patient: Matthew James Channon, DOB [REDACTED] 1997

Date and Time: 5/8/2026 2:00 PM - 3:00 PM

Duration: 60 minutes

Service Code: 6016

Location: Main Office

Participants: Client only

Presenting Problem

The client was referred to the agency today for assessment. He has a history of two hospitalizations for observation due to hyponatremia, which is defined as low sodium levels in the blood. Following this, he was transferred to another facility for an 11-day evaluation by a psychiatrist. The psychiatrist initially prescribed Seroquel; however, during a follow-up appointment, he informed the client that the medication was not necessary. The client denies any history of substance abuse, including alcohol or illicit drugs, and reports adherence to prescription medications as directed by his healthcare providers.

Current Mental Status

Orientation:	X3: Oriented to Person, Place, and Time
General Appearance:	Appropriate
Dress:	Appropriate
Motor Activity:	Unremarkable
Interview Behavior:	Appropriate
Speech:	Normal
Mood:	Euthymic
Affect:	Congruent
Insight:	Fair
Judgment/Impulse Control:	Fair
Memory:	Intact
Attention/Concentration:	Good
Thought Process:	Unremarkable
Thought Content:	Appropriate
Perception:	Unremarkable
Functional Status:	Intact

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Objective Content

additional topics discussed during session include: Informed consent, and policies related program attendance and communication with agency staff.

Biopsychosocial Assessment

Identification:	Client is a 49 year old White male. He was married at one point and shares custody of a 12 year old son.
History of Present Problem:	Client stated he experiences occasional bouts with depression due to low levels of testosterone. He did not specify if he takes medication for this condition.
Psychiatric History:	Client stated he was sent to a psychiatric facility for observation and was prescribed Seroquel by the psychiatrist, upon a follow up appointment he was told to discontinue the



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Trauma History: Seroquel. He no longer meets with a psychiatrist.
Family Psychiatric History: Client stated in the past being involved in four auto accidents.
Medical Conditions & History: Client is not aware of any family psychiatric history.
Substance Use: Client has low testosterone cause him to become depressed. He is taking a GLP for obesity, and lives with sleep apnea.
Family History: Client denies any substance abuse.
Social History: Client has a 12 year old son for which he co- parents.
Spiritual/Cultural Factors: Client stated is marriage to the mother of his son ended in 2024.
Developmental History: Client denies any spiritual or cultural factors.
Educational/Vocational History: Client denies any negative developmental history.
Legal History: Client has Master's Degree in Electrical Engineering.
SNAP: Client states that his current legal problems are his only issues with the law.
S- I think I'm very resilient, patience
N- Someone to talk to
A- " I can whistle very well, writes app for phone, good in Traveling
P- Individual session if appropriate.

Medications

- GLP-1
- Testosterone

Plan

next step in the treatment process includes further evaluation if appropriate, and the development of a full treatment plan.

Diagnosis

F06.31 Depressive Disorder Due to Another Medical Condition, With depressive features

Diagnostic Justification

Client states his depressive symptoms are exaggerated when he is not taking his testosterone medication. Client reports the following symptoms: Lack of motivation; Little interest in things he once had interest in; and sleeping too much or too little.

Treatment recommendation: 6012 4 units Mental Health Counseling

Martha L Sanchez, LMSW, Licensed Social Worker, License NM M-10875, signed this note and declared this information to be accurate and complete on 5/31/2026 at 9:10 AM.

Kirsten Hockemeier, Licensed Clinical Social Worker, License NM C-3387, signed this note and declared this information to be accurate and complete on 6/1/2026 at 7:21 AM.